

Discharge Summary

Patient: Friedrich Kremer

Date of Birth: 10/23/1957

Admission Date: 10/14/2024

Discharge Date: 10/21/2024

Attending Physician: Dr. Annette Hoffmann, MD

Dear Dr. Meier,

We are writing to inform you at length about the inpatient stay of our mutual patient, Mr. Friedrich Kremer, born October 23, 1957, admitted October 14 to October 21, 2024.

Mr. Kremer was admitted due to suboptimal diabetic control. On admission, HbA1c was significantly elevated at 9.8%, indicating a prolonged hyperglycemic state over several months. Signs of early peripheral arterial disease were also identified.

During the inpatient stay, a comprehensive review of the treatment plan was undertaken. The insulin regimen was adjusted and a structured diabetes education program was completed.

Primary Diagnosis: Type 2 diabetes mellitus with peripheral angiopathy (E11.5)

We request an outpatient follow-up in 6 weeks and repeat HbA1c in 3 months.

Relevant Medical History:

The patient has a long-standing history of arterial hypertension diagnosed in 2008, currently managed with an ACE inhibitor and a calcium channel blocker. Type 2 diabetes mellitus was diagnosed in 2015 and is managed with oral antidiabetic agents and insulin. Additionally, the patient has a history of hypercholesterolemia and is on statin therapy. There is no known history of malignancy, significant surgical procedures, or major allergies.

Family history is notable for coronary artery disease in the patient's father, who suffered a myocardial infarction at age 62. The patient is a former smoker with a 20 pack-year history, having ceased smoking in 2010. Alcohol consumption is reported as moderate.

Follow-up Recommendations:

We recommend the following follow-up measures: (1) General practitioner visit within 2 weeks for medication review and blood pressure control. (2) Laboratory workup including complete blood count, renal function, liver enzymes, and HbA1c in 3 months. (3) Specialist referral as outlined above. (4) Repeat imaging in 6 months if clinically indicated.

The patient has been fully informed of the diagnosis, treatment plan, and recommended follow-up schedule. Written discharge instructions were provided. The patient expressed understanding and willingness to comply with the recommended measures.

Medication on Discharge:

The following medications were prescribed at discharge: metformin 1000 mg twice daily, ramipril 5 mg once daily, atorvastatin 40 mg once daily, aspirin 100 mg once daily, and pantoprazole 40 mg once daily. The patient was counseled on the importance of adherence and potential side effects.

Dr. Annette Hoffmann, MD

Senior Physician, Internal Medicine